



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : سوزان محمد ابوزيد سعيد
المحافظة : القاهرة
الحى / المدينة : الشراييه
رقم البطاقة : 25907300102085 رقم التليفون : 01214341222
العمر (تاريخ الميلاد) : 30-07-1959 المهنة : ربه منزل
العنوان : 21 ش حافظ صلاح
تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي الفقرات القطنية بالصبغة
قسم العلاج :
الأورام

[12771]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: سوزان محمد أبو زيد سعيد
National ID: 25907300102085



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced MRI of the lumbar spine

الوقت :

-- --

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بمطابقة تحقيق الشخصية



الدولان

محمد أبو زيد محمد

٢١ شارع حافظ صالح

الشرابية - القاهرة

١٩٩١/١٢

٢٥٩.٧٣.٠١.٢.٨٥

IH0524468

ارل

حرة
٢٢٢١٤١٩



٢٠١٩/١٢ ٢٥٩.٧٣.٠١.٢.٨٥

مدير عام العلاقات العامة بالمتاح

اتحاد الأندية والتلفزيون

أنثى مسجلة

كامل حسام كامل محمد

البطاقة مصرية حتى ٢٠٢٩/١٢/٢٥



HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION SYSTEM AND DIGITAL
TRANSFORMATION



الهيئة العامة للتأمين الصحي
الإدارة المركزية لتنظيم المعلومات و التحول
الرقمي

رسوم الاستعلام 20 جنية

جهة الاصدار : فرع القاهرة

تاريخ الاصدار : 2026-8-8

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للاقتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل" و كذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .

وطبقا لقاعدة البيانات المرسله من التأمينات الاجتماعيه

وطبقا لقاعدة البيانات المرسله من الضمان الاجتماعي

والبحث تبين أن

السيد (6) سوزان محمد ابو زيد بطاقة رقم 25907300102085

تليفون رقم: 01214341222

وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الاقتفاع
يعتبر من الفئات غير المتفعله بالتأمين الصحي صدر هذا الخطاب دون ادنى مسئولية على الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعة : 2026-8-8

الموظف المختص

محمد مصطفى

إفادة مركز نظم المعلومات
بمعمد...

مسلسل 2043991



الخطاب ساري لمدة ثلاثة اشهر من تاريخ الاصدار الا في حالة تغيير حالة المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو
وزارة التأمين الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية لتنظيم المعلومات و التحول الرقمي

Name: سوزان محمد أبو زيد سعيد		
MR. : 13132		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 67-year-old individual who is post-menopausal. There is no reported family history of breast cancer and the only noted comorbidity is treatment with ELTROXIN 100. The core biopsy demonstrated invasive ductal carcinoma, grade 2 (IDC G2). Immunohistochemistry showed estrogen receptor 0/8, progesterone receptor 3/8, and HER2 scored +3. Contrast-enhanced spectral mammography (CESM) revealed a right-upper-outer quadrant mass and non-mass enhancement measuring 8 × 5.3 × 4.4 cm, with right-level 1 and several enlarged lymph nodes. Staging CT of the chest, abdomen, and pelvis identified a right hepatic focal lesion with biphasic criteria consistent with a hemangioma. Bone scintigraphy demonstrated a few active osseous lesions in the sacrum, left ischium, and left femoral trochanteric region, pending CT correlation. The referring team recommended radiology correlation of the CT findings; if correlation is not achieved, neoadjuvant chemotherapy (NACT) with clipping followed by radiotherapy, wide local excision, and lymph node management according to response. The multidisciplinary team approved portacath insertion, MRI of the sacrum and left femur, placement of clips, and initiation of paclitaxel together with Herceptin.		
تحريراً في ,		
	التاريخ :	23-09-2026
	الوقت :	
	الطبيب المعالج :	
يعتمد ,		

MRN:	10126283077	D.O.B:	30/07/1959
Patient Name:	SUZAN MOHAMMED ABOZEID SAYED	Sex:	Female
IC No.:	25907300102085		
Location:	External Clinic		

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2607200218	Collection Date	20/07/2026 02:13 PM
Accession No	SUR-26-2544	Received Date	20/07/2026 02:13 PM
Sample Type	Tissue	Doctor	Omnia ASHRAF HASSAN ALAAM

Clinical Information

Right breast lesion at 9-10 O'clock (BIRADS 4c).

Gross Picture

Multiple grayish tissue cores 1-1.5 cm long, totally submitted.

Microscopic Picture

A case of invasive duct carcinoma, grade 3. There is an associated high grade DCIS component with comedo necrosis & calcifications. Lymphovascular emboli are detected. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, NST, grade 3.

Laterality

Right

Specimen type:

U/S guided core biopsy.

Location

9-10 O'clock

Lesion Size

Invasive tumor size/s (mm):

Total extent of lesion (mm):

Multiple Lesions

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3

Histological grade (1-3):

3

Peritumoural lymphovascular invasion

Present

Involvement of skin

Involvement of muscle

DCIS in specimen

Present

Percentage

Nuclear grade:

Intermediate

Necrosis:

Architecture:

Solid

Calcification:

Yes

Diagnosis

Right breast lesion, U/S guided core biopsy:

Invasive duct carcinoma, grade 3.

Comments

ER, PR & HER-2 are available(upon request).

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Ahmed Ali. MD.

Co-signed by:

Dr.Mahitab Eltohamy ,MD

Pathology Department

قسم الباثولوجي

ب. هيا

MRN	10126283077	D.O.B	30/07/1959
Patient Name	SUZAN MOHAMMED ABOZEID SAYED	Sex	Female
IC No.	25907300102085		
Location	External Clinic		

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2607280232
Accession No IMM-26-3142
Sample Type Paraffin Block

Collection Date 29/07/2026 02:41 PM
Received Date 29/07/2026 02:41 PM
Doctor Mahmoud sayed Abdelmonsen Meshref

Immunohistochemical Report

pathology number
2644/26

Clinical Data
Right Breast cancer

Gross Picture:
Infected block submitted for ER/PR immunostaining.

System Used:
Ventana BenchMark ULTRA Autostainer.

Antibody:
Ventana anti-ER[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody

Scoring system:
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Staining score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
There are much more limited data on the 2% to 3% of cancers that are low ER expressers most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

Generated on 30/07/2026 03:00 PM

Print Date..

04/08/2026 02:39 PM

Page Number: 1 of 2



www.Baheya.org



16602

مركز بيهة الهرم
4 شارع غلوبة اول شارع الهرم بجوار دارى , الحيزه
مستشفى بيهة الشبلخ زايد
ع. ابي الاول - الشيخ زايد امام كلية الهندسة



[doctype=html]

ER positive in 1-20% of tumor cells is considered as Low positive.

Immunohistochemistry: Breast Cancer

Right

Diagnosis:

ER: Negative (0/8)

PR: positive, weak nuclear reaction in 2% of tumor cells (3/8).

Signature

Dr. Al Shimaa Zakaria, MD

Reported and electronically endorsed by
El Shimaa Zakaria Fathy Ali

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

Collected on: 30/07/2026 03:00 PM

Print Date

04/08/2026 02:39 PM

Page Number: 2 of 2



www.Baheya.org



16602

مركز رعاية الهرم
دار طبقات شارع الهرم بجوار نادي الجزيرة



مستشفى رعاية الشيخ زايد
الحي الأول - الشيخ زايد - مدينة خليفة



Name: Mrs. Suzan Mohamed Abou Zaid Saied

Date: 16/7/2026

ID:9926012749

Acc.:240326998

Computed soft tissue mammography with superficial ultrasound examination of both breasts, revealed:

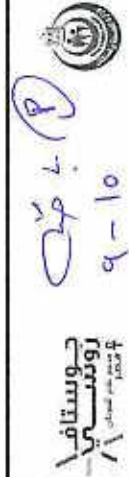
Indication: Right breast palpable mass.

Right breast:

- Heterogeneous dense breast (ACR.C).
- Right breast: UOQ an ill-defined dense mass lesion with overlying suspicious micro-calcifications, measure 4 x 3.5 cm, away from nipple by 4 cm in CC view, by ultrasound: at 10 o'clock: an ill-defined hypoechoic lesion with posterior acoustic shadowing surrounded with desmoplastic reaction, with internal vascularity on color Doppler, measure 29 x 17 x 31 mm, away from overlying skin by 3 mm.
- Multiple very tiny similar lesions see surrounded the main lesion / multi focal.
- Normal skin thickness and contour.
- Normal nipple/areola complexes.
- Right pathologically enlarged lymph nodes, more than three nodes (level 1 and 2), the largest one measure 21 x 15 mm.

Left breast:

- Heterogeneous dense breast (ACR.C).
- Normal both breast parenchyma with no evidence of architecture distortion or speculated lesions seen.
- Left breast: retroareolar: partially well circumscribed partially obscure margin iso dense lesion by ultrasound: retroareolar: two well defined macro lobulated hypoechoic lesions, seen parallel to breast parenchyma, with no internal vascularity on color Doppler, one measure 19 x 7 x 18 mm, another one measure 5 x 4 mm.
- No suspicious micro-calcific clusters noted.
- breast parenchyma appear homogenously hyperechoic with prominent glandular element /mottled pattern
- Normal skin thickness and contour.
- Normal nipple/areola complexes.
- Left non-specific axillary lymph nodes.



مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الهيئة العامة
للتنظيم
الحياتي

09/15/2026

MDT FORM

Name:	سوزن محمد فوزية محمد	MRN:	13132
Age:	67 Y	ID:	25907300102085
Menopausal Sta:	POST	Phone:*	01022222997
NRS SCORE:	PS 1	Marital Status:	MARRIED
BMI:	25	Family History:	-VE
Gender		Reason Of MDT:	NEW
CO Morbidity:	ELTROXIN 100	ECOG Performance Status	0 0 1 0 2 0 3 0 4
Surgical History:*	TOTAL THYROIDECTOMY LAP CHOLECYSTECTOMY PUH +MESH APPENDLECTOMY		

Clinical Data:

BX: IDC G2
ER: 0/8 PR: 3/8 HER2: +3 KI67:

History:

Sonomammography:

CESM
RT MASS AND NON MASS ENHANCEMENT IN UOQ 8X5.3X4.4CM
RT LEVEL 1 AND FEW LNS >3 ENLARGED

Biomarker/Molecular

Mets Workup:

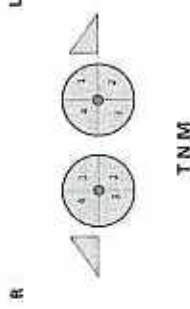
CT CAP ///RT HEPATIC FOCAL LESION WITH BIPHASIC CRITERIA OF THE
HEMANGIOMA
BONE SCAN>>> FEW ACTIVE OSSEOUS LESIONS AT SCARUM LT ISCHIUM
AND TROCHANTRIC OF LT FEMUR FOR CT CORRELATION

Recommendations:

FOR CORRELATION OF CT BY RADIOLOGY CONSULT
IF NOT MET, FOR NACT AND CLIPPING THEN RT WLE AND LN ACCORDING
TO RESPONSE

MDT Outcome & Plan

- Portacath Insertion
- MRI of the sacrum + left femur
- Clips Insertion
- Start Paclitaxel - Herceptin



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

T2 N1 Mx

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. خالد عبد العزيز	د. محمد فوزي		



مستشفى الزهراء الجامعي

قسم القلب والأوعية الدموية
وحده الموجات فوق صوتيه للقلب

Echocardiogram Report

NAME	سوزان محمد أبوزيد	Age	Female/ 67 Y
Operator	Dr. Abdallah Ahmed Abdallah <i>A lecturer of cardiology</i>		

M-Mode & Doppler Data:

Dimensions	Estimated	Normal	Dimensions	Estimated	Normal
IVS	9	11 mm	EF	67	>55 %
LVd	47	56 mm	FS	36	
LVs	30	45 mm	AO	33	37 mm
PW	10	11 mm	LA	31	40 mm

Interpretation:

Left Ventricle	- Normal internal dimension, systolic function and overall contractility. - <u>Impaired diastolic function (mild grade I).</u>
Wall Motion	- No gross segmental wall motion abnormalities.
Left atrium	- Normal.
Right atrium	- Normal.
Right Ventricle	- Normal dimension and normal systolic function.
Mitral valve	- <u>Normal with mild regurgitation.</u>
Aorta & valve	- Normal root dimension. - Normal tricusps valve.
Tricuspid valve	- <u>Normal with trivial regurgitation, estimated PASP= 26 mmHg.</u>
Pulmonary & valve	- Normal trunk and valve
Septa	- Intact.
pericardium	- No effusion.
Masses	- No masses or thrombi.

Impression:

**Impaired LV diastolic function (mild grade I)
mild MR, otherwise normal finding**

12/08/2026 10:19 AM

Name : Suzan Mohammed Abu Zeid Said

Date : 05-09-2026 07:45

Branch : Dokki

Medical No. : 454441

Ref.Physician Prof : Goustav Roussey Egypt

R
F
T
N
F
C

C
I
T
S
O
N
G
A
I
D
F
L
I
N

Tc-99m MDP SKELETAL SCINTIGRAPHY

History

67 year-old female patient, who gave history of right breast cancer, (previous history of trauma and fracture right ? arm and lumbar vertebrae), she complains of bone ache and referred for skeletal survey.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Enhanced radiotracer uptake is noticed at the head of the right humerus, likely attributed to previous trauma/fracture.
- Focal active osseous lesions are noted at the sacrum, left ischium and trochanteric region of the left femur.
- Degenerative changes are noted at different levels of the spine and around big joints.
- Otherwise rest of the scanned skeleton is unremarkable.

IMPRESSION

- Few active osseous lesions at the sacrum, left ischium and trochanteric region of the left femur warranting CT correlation to clarify its nature.
- Rest of the scanned skeleton is unremarkable .

Much obliged

**Dr. Alaa Nagy, MD,
Nuclear medicine**

Name: Mrs. Suzan Mohamed Abou Zaid Saied

Date: 16/7/2026

ID:9926012749

Acc.:240326998

Computed soft tissue mammography with superficial ultrasound examination of both breasts, revealed:

Indication: Right breast palpable mass.

Right breast:

- Heterogeneous dense breast (ACR.C).
- Right breast: UOQ an ill-defined dense mass lesion with overlying suspicious micro-calcifications, measure 4 x 3.5 cm, away from nipple by 4 cm in CC view, by ultrasound: at 10 o'clock: an ill-defined hypoechoic lesion with posterior acoustic shadowing surrounded with desmoplastic reaction, with internal vascularity on color Doppler, measure 29 x 17 x 31 mm, away from overlying skin by 3 mm.
- Multiple very tiny similar lesions see surrounded the main lesion / multi focal.
- Normal skin thickness and contour.
- Normal nipple/areola complexes.
- Right pathologically enlarged lymph nodes, more than three nodes (level 1 and 2), the largest one measure 21 x 15 mm.

Left breast:

- Heterogeneous dense breast (ACR.C).
- Normal both breast parenchyma with no evidence of architecture distortion or speculated lesions seen.
- Left breast: retroareolar: partially well circumscribed partially obscure margin iso dense lesion by ultrasound: retroareolar: two well defined macro lobulated hypoechoic lesions, seen parallel to breast parenchyma, with no internal vascularity on color Doppler, one measure 19 x 7 x 18 mm, another one measure 5 x 4 mm.
- No suspicious micro-calcific clusters noted.
- breast parenchyma appear homogeneously hyperechoic with prominent glandular element /mottled pattern
- Normal skin thickness and contour.
- Normal nipple/areola complexes.
- Left non-specific axillary lymph nodes.

MRN: 10126283077
Patient Name: SUZAN MOHAMMED ABOZEID SAYED
IC No.: 25907300102085
Location: External Clinic

D.O.B: 30/07/1959
Sex: Female

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2607200218	Collection Date	20/07/2026 02:13 PM
Accession No	SUR-26-2544	Received Date	20/07/2026 02:13 PM
Sample Type	Tissue	Doctor	Omnia ASHRAF HASSAN ALAAM

Clinical Information

Right breast lesion at 9-10 O'clock (BIRADS 4c).

Gross Picture

Multiple grayish tissue cores 1-1.5 cm long, totally submitted.

Microscopic Picture

A case of invasive duct carcinoma, grade 3. There is an associated high grade DCIS component with comedo necrosis & calcifications. Lymphovascular emboli are detected. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, NST, grade 3.

Laterality

Right

Specimen type:

U/S guided core biopsy.

Location

9-10 O'clock

Lesion Size

Invasive tumor size/s (mm):

Total extent of lesion (mm):

Multiple Lesions

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3

Histological grade (1-3):

3

Peritumoural lymphovascular invasion

Present

Involvement of skin

Involvement of muscle

DCIS in specimen

Present

Percentage

Nuclear grade:

Intermediate

Necrosis:

Architecture:

Solid

Calcification:

Yes

Diagnosis

Right breast lesion, U/S guided core biopsy:

Invasive duct carcinoma, grade 3.

Comments

ER, PR & HER-2 are available(upon request).

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Ahmed Ali. MD.

Co-signed by:

Dr.Mahitab Eltohamy ,MD



MRN: 10126283077

Patient Name: SUZAN MOHAMMED ABOZEID SAYED

IC No.: 25907300102085

Location: External Clinic

D.O.B: 30/07/1959

Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id	2607290231	Collection Date	29/07/2026 02:41 PM
Accession No	IMM-26-3141	Received Date	29/07/2026 02:41 PM
Sample Type	Paraffin Block	Doctor	Mahmoud sayed Abdelmohsen Meshref

Immunohistochemical Report

pathology number
2544/26

Clinical Data

Right breast cancer

Gross Picture :

Referred block submitted for HER 2 immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+ : The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+ : based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:

Right

Her2 Result

Positive [score 3+]

Signature

Dr. Al Shimaa Zakaria, MD



MRN: 10126283077
 Patient Name: SUZAN MOHAMMED ABOZEID SAYED
 IC No.: 25907300102085
 Location: External Clinic

D.O.B: 30/07/1959
 Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2607290232
 Accession No IMM-26-3142
 Sample Type Paraffin Block

Collection Date 29/07/2026 02:41 PM
 Received Date 29/07/2026 02:41 PM
 Doctor Mahmoud sayed Abdelmohsen Meshref

Immunohistochemical Report

pathology number
2541/26

Clinical Data
Right breast cancer

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:

There are much more limited data on the 2% to 3% of cancers that are low ER expressers most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

Validated on 30/07/2026 03:00 PM

Print Date: 04/08/2026 02:39 PM

Page Number: 1 of 2



www.Baheya.org



16602

مركز بهيّة الهرم
4 شارع طلحة اول شارع الهرم بجوار داري العوجة

مستشفى بهيّة الشيخ زايد
1. الحي الاول - الشيخ زايد شارع كلية الهندسة

Patient Name	سوزان محمد ابوزيد	Case No.	357649
Sex/Age	Female 67 Y	Patient No.	25800
Registered Date	12/8/2026 11:43:55 AM	Referred By Dr.	Out Patient
Validation Date	20/8/2026 09:25:25 AM	Branch	Gustave Roussy
Reporting Date	27/8/2026 10:20:07 AM	Pathology No.	SR-931/26

Breast Slide Revision

Clinical Data: Right breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (2544/26). Together with, four referred slides including 1 (H&E) stained slide and 3 immunostained slides for (ER, PR& Her2).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: present, high grade, solid with comodo necrosis.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Microcalcification: Absent.
- * Moderate tumor infiltrating lymphocytes.

IPT:

- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: Weak nuclear reaction in 5% of tumor cells (score 3/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):
 - Invasive micropapillary carcinoma, Grade 3.

IPT:

- ER: Negative (0/8).
- PR: Positive (3/8).
- Her2: High (score 3+).

* *Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%, 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987, Nile Corniche - Alser El Qadema - Cairo, Egypt

Patient Name	سوزان محمد ابوزيد	Case No.	357649
Sex/Age	Female 67 Y	Patient No.	25800
Registered Date	12/8/2026 11:43:55 AM	Referred By Dr.	Out Patient
Validation Date	20/8/2026 09:25:25 AM	Branch	Gustave Roussy
Reporting Date	27/8/2026 10:20:07 AM	Pathology No.	SR-931/26

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+; as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+; based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+; based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim, MD.



غرفة رقم (4) - الطور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor- Building No. 987- Nile Corniche - Alse El Qadima - Cairo, Egypt

اسم المريض بالإنجليزية: suzan mohamed abozeid	كود المريض: 25907300102085
تاريخ التقرير: 2026/08/20 10:50:18 AM	المرضى: سنة ٤٧

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 67 years old female patient with pathologically proven invasive ductal carcinoma grade II (-ve ER, -ve PR and +ve HER2)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Mild nodular background enhancement.

Heterogeneously dense

Right breast:

- **Low energy (Non contrast) images:** Right breast shows middle 1/3 UOQ irregular shaped spiculated dense lesion with foci of microcalcifications within associated with architectural distortion.

- **Recombined (Post contrast) images:**

- Right breast shows focal malignant process inform of large UOQ middle 1/3 irregular spiculated heterogeneous moderate mass enhancement and measuring about 3.2 x 3.3 x 3.3 cm at its AP, TS and CC diameters and about 4.4 cm from the nipple and about 3.7 mm from the skin

- Associated anterior and posterior moderate heterogeneous non mass enhancement extension are noted at the anterior and posterior 1/3s UOQ and collectively measuring overall about 8.0 x 5.3 x 4.4 cm at its AP, TS and CC diameters and about 3 cm from nipple.

- **Complementary US:**

- The right breast shows large irregular shaped spiculated hypoechoic lesion located at 10:00 clock position surrounded by anterior and posterior ill-defined area of altered parenchyma

- Few enlarged axillary lymph node at level I and II (>3) with focal cortical thickening, one of them show irregular margins measuring about 22.6 x 16 mm.

Left breast:

- **Low energy (Non contrast) images:** the left breast shows anterior 1/3 retroareolar partially circumscribed partially obscured dense mass lesion measuring about 19 x 13 mm.

- **Recombined (Post contrast) images:** no pathological uptake.

- **Normal skin / nipple / areolar complex.**

- **Complementary ultrasound:**

- Left breast shows retroareolar rather well defined hypoechoic oval shaped mass lesion measuring about 19 x 13 mm.

- No enlarged axillary L.Ns.

IMPRESSION:

- **RIGHT BREAST:** (BIRADS 6= pathological proven) Right UOQ moderate

heterogeneous mass and non-mass enhancement as described above.

- **LEFT BREAST:** (BIRADS 3) left retroareolar probably benign non-enhancing mass likely fibroadenoma.

Best regards,

Dr. Hedaya Nour, MD

Patient's Name: Suzan Mohammed Abu Zeid Said
Date: 2026-08-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. A right hepatic lobe segment VI focal lesion measuring 76 x 54 mm showed nodular peripheral enhancement with filling in of contrast in the delayed phase.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulac or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ **Right hepatic lobe focal lesion with biphasic criteria of the hemangioma for dynamic MRI.**

Dr.Assem Ahmed



مركز النيل للأشعة والتحاليل

Name : Suzan Mohammed Abu Zeid Said	Date : 24-09-2026 07:37
Branch : Nasrcity	Medical No. : 454441
Ref. Physician Prof : Ahmed Gab Allah	Ins. Company : Walkin

Whole body (FDG) PET /CT Study:

Clinical history:

- 67-year-old female patient with history of right breast cancer.
- PET/CT is requested for status evaluation.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV, a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 3.0

PET/CT findings:

Head and neck:

- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.
- No pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.

Chest:

- Metabolically active right breast lesions are noted, the largest measuring 2.1 cm with SUV max 4.7.
- Metabolically active axillary nodal lesions, measuring 12 mm with SUV max 5.5.
- No current significant metabolically active lung nodules or parenchymal infiltrates.
- No enlarged or tracer-avid mediastinal, hilar or left axillary LN.s.
- No pleural thickening, masses or effusion.

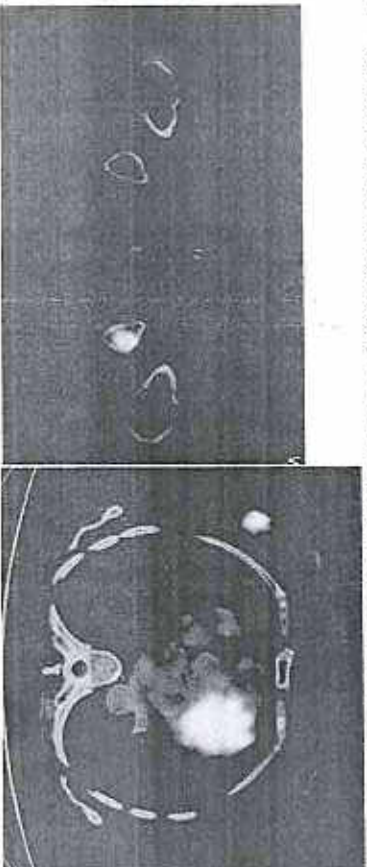
Abdomen and Pelvis:

- Tracer starved large right hypodense hepatic lesion is noted measuring 7.5 cm at segment VI , likely hemangioma for follow up.
- Otherwise, no current significant hypermetabolic hepatic focal lesions detected.

Name : Suzan Mohammed Abu Zeid Said
Branch : Nasreity
Ref.Physician Prof : Ahmed Gab Allah

Date : 24-09-2026 07:37
Medical No. : 454441
Ins.Company : Walkin

- Unremarkable spleen, kidneys, adrenals, pancreas and pelvic organs.
- No definite gastric or bowel thickening, mural masses (on non-oral contrast CT basis) or tracer-avid foci could be detected.
- No pathologically enlarged or tracer-avid abdomino-pelvic LN.s.
- No evidence of ascites, free or encysted abdomino-pelvic fluid collections.



Musculoskeletal & BM:

- Metabolically active osseous lesions are noted at LV3, sacrum and left pubic bone with SUV max 12.5.

Impression:

Whole-body (FDG) PET/CT study is showing:

- Metabolically active neoplastic right breast lesions associated with right axillary nodal and osseous lesions as described.
- The rest of the surveyed body show no scintigraphic evidence of any other significant metabolically active lesions.

Best regards,

Dr. Ahmed dowdyar, MSc.

Dr. Alaa Nagi, MD.
Nuclear Medicine

اسم المريض بالإنجليزية: Sozan Mohamed Abo Zaid	كود المريض: 101004
تاريخ التقرير: 2026/09/30 11:59:55 AM	اللقب: ٢٧ سنة ٢ شهر

Women's Health Outreach Program
Ultrasound guided clip insertion

Right breast

- Under complete aseptic condition & ultrasound guidance two clip insertion was done for right breast UOQ pathologically proven malignant process (two malignant mass lesions at 10 & 11 o'clock with surrounding altered parenchyma correlate to mass & surrounding non-mass enhancement of the CESM)
- One clip was inserted at the outer & inner edges of altered parenchyma, both masses were within such marked area.
- This was confirmed by mammographic CC & ML views.

Best Regards,
Dr. Amal Hegazi, M.Sc.